

SENATE No. 3203

Senate, July 23, 2026 -- Text of amendment (64) (offered by Senator Creem) to the Ways and Means amendment (Senate, No. 3178) to the House Bill relative to economic development in the commonwealth.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

1 by adding the following sections:-

2 "SECTION X. Section 47H of chapter 175, as appearing in the 2022 Official Edition, is
3 hereby amended by striking out the final two sentences and inserting in place thereof the
4 following:-

5 For purposes of this section, “infertility” means a condition or status characterized by any
6 of the following:

7 (1) A licensed physician’s findings, based on: a patient’s medical, sexual, and
8 reproductive history; age; physical findings; diagnostic testing; or any combination of those
9 factors. This definition shall not prevent testing and diagnosis of infertility to establish infertility
10 with or without appropriate exposure to gametes, per the patient’s provider.

11 (2) The need for medical intervention, including, but not limited to, the use of donor
12 gametes, donor embryos, gestational carrier to achieve a live birth either as an individual or with
13 a partner.

(3) The failure to establish a pregnancy or to carry a pregnancy to live birth after unprotected sexual intercourse. For purposes of this section, “unprotected sexual intercourse” means no more than 12 months of unprotected sexual intercourse for a person under 35 years of age or no more than 6 months of unprotected sexual intercourse for a person 35 years of age or older. Pregnancy that does not result in a live birth will not restart the 12-month or 6-month time period to qualify as having infertility.

(4) An impairment of reproductive ability due to factors, including, but not limited to, medical condition, male factor, female factor, combined or unexplained reproductive challenges, as well as genetic disorders or iatrogenic infertility.

Coverage for medically necessary expenses of diagnosis and treatment of infertility shall include, but shall not be limited to: (i) a minimum of six oocyte retrievals and unlimited fresh and frozen embryo transfers, using single embryo transfer when recommended by patient’s physician and medically appropriate; (ii) embryo transfer; (iii) artificial insemination; (iv) surgical sperm extraction procedures; (v) third-party reproduction including in vitro fertilization with donor egg, sperm, or embryo or gestational carrier; (vi) procedures necessary to screen or diagnose a fertilized egg before transfer, including, but not limited to, preimplantation genetic testing for aneuploidy, preimplantation genetic testing for chromosome structural rearrangements, and preimplantation genetic testing for monogenic or single gene disorders.

In administering coverage for medically necessary expenses of diagnosis and treatment of infertility, a carrier or participating provider, as those terms are defined in section 1 of chapter 176O, shall not:

(1) impose conditions for eligibility beyond what is provided in the law;

(2) exclude, limit, or otherwise restrict coverage or processing of benefits for fertility medications that are different from those imposed on other prescription medications;

(3) exclude or deny coverage of any fertility services, including medication, based on an individual's participation in fertility services provided by or to any third party. For purposes of this paragraph, "third party" includes: (i) any fresh or cryopreserved oocyte, sperm, or embryo, regardless of the initial coverage source of the donor or the genetic material; and (ii) a gestational carrier that enables an intended parent, member, and/or partner of a member to become a parent.

(4) exclude services based on the quantity of the patient's existing cryopreserved oocyte, sperm, or embryos; the provider's discretion will determine if cryopreserved oocyte, sperm, or embryo provides a reasonable chance of success and whether additional fertility services are required;

(5) implement any deductible, copayment, coinsurance, benefit maximum, waiting period, or other limitation on coverage that is different from those imposed upon benefits for services not related to infertility;

(6) impose limitations on coverage based solely on arbitrary, non-medically based factors including, but not limited to, number of attempts, dollar amounts, or age; or

(7) provide different benefits to, or impose different requirements for different groups, based on diagnosis.

Limitations on coverage coverage for medically necessary expenses of diagnosis and treatment of infertility shall be based on clinical guidelines and the patient's medical history.

Clinical guidelines shall be maintained in written form and available to any enrollee. Standards

or guidelines developed by the American Society for Reproductive Medicine, the American College of Obstetrics and Gynecology, the Society for Assisted Reproductive Technology, or similar relevant medical societies may serve as a basis for such clinical guidelines. Making, issuing, circulating, or causing to be made, issued or circulated, any clinical guidelines that are based upon data that are not reasonably current or that do not cite with specificity any references relied upon shall constitute an unfair and deceptive act and practice pursuant to section 2 of chapter 93A.

Consistent with Massachusetts anti-discrimination law, coverage for medically necessary expenses of diagnosis and treatment of infertility shall be provided without discrimination based on age, ancestry, color, disability, domestic partner status, gender, gender expression, gender identity, genetic information, marital status, national origin, race, religion, sex, or sexual orientation.

This section shall not be construed to deny or restrict any existing right or benefit to coverage and treatment of infertility or fertility services under an existing law, plan, or policy. This section shall not be construed to interfere with a medical provider's, physician's, or surgeon's clinical judgment.

SECTION X. Section 8K of chapter 176A, as appearing in the 2022 Official Edition, is hereby amended by striking out the final two sentences and inserting in place thereof the following:-

For purposes of this section, "infertility" means a condition or status characterized by any of the following:

(1) A licensed physician's findings, based on: a patient's medical, sexual, and reproductive history; age; physical findings; diagnostic testing; or any combination of those factors. This definition shall not prevent testing and diagnosis of infertility to establish infertility with or without appropriate exposure to gametes, per the patient's provider.

(2) The need for medical intervention, including, but not limited to, the use of donor gametes, donor embryos, gestational carrier to achieve a live birth either as an individual or with a partner.

(3) The failure to establish a pregnancy or to carry a pregnancy to live birth after unprotected sexual intercourse. For purposes of this section, "unprotected sexual intercourse" means no more than 12 months of unprotected sexual intercourse for a person under 35 years of age or no more than 6 months of unprotected sexual intercourse for a person 35 years of age or older. Pregnancy that does not result in a live birth will not restart the 12-month or 6-month time period to qualify as having infertility.

(4) An impairment of reproductive ability due to factors, including, but not limited to, medical condition, male factor, female factor, combined or unexplained reproductive challenges, as well as genetic disorders or iatrogenic infertility.

Coverage for medically necessary expenses of diagnosis and treatment of infertility shall include, but shall not be limited to: (i) a minimum of six oocyte retrievals and unlimited fresh and frozen embryo transfers, using single embryo transfer when recommended by patient's physician and medically appropriate; (ii) embryo transfer; (iii) artificial insemination; (iv) surgical sperm extraction procedures; (v) third-party reproduction including in vitro fertilization with donor egg, sperm, or embryo or gestational carrier; (vi) procedures necessary to screen or

100 diagnose a fertilized egg before transfer, including, but not limited to, preimplantation genetic
101 testing for aneuploidy, preimplantation genetic testing for chromosome structural
102 rearrangements, and preimplantation genetic testing for monogenic or single gene disorders.

103 In administering coverage for medically necessary expenses of diagnosis and treatment of
104 infertility, a carrier or participating provider, as those terms are defined in section 1 of chapter
105 176O, shall not:

106 (1) impose conditions for eligibility beyond what is provided in the law;

107 (2) exclude, limit, or otherwise restrict coverage or processing of benefits for fertility
108 medications that are different from those imposed on other prescription medications;

109 (3) exclude or deny coverage of any fertility services, including medication, based on an
110 individual's participation in fertility services provided by or to any third party. For purposes of
111 this paragraph, "third party" includes: (i) any fresh or cryopreserved oocyte, sperm, or embryo,
112 regardless of the initial coverage source of the donor or the genetic material; and (ii) a gestational
113 carrier that enables an intended parent, member, and/or partner of a member to become a parent.

114 (4) exclude services based on the quantity of the patient's existing cryopreserved oocyte,
115 sperm, or embryos; the provider's discretion will determine if cryopreserved oocyte, sperm, or
116 embryo provides a reasonable chance of success and whether additional fertility services are
117 required;

118 (5) implement any deductible, copayment, coinsurance, benefit maximum, waiting
119 period, or other limitation on coverage that is different from those imposed upon benefits for
120 services not related to infertility;

(6) impose limitations on coverage based solely on arbitrary, non-medically based factors including, but not limited to, number of attempts, dollar amounts, or age; or

(7) provide different benefits to, or impose different requirements for different groups, based on diagnosis.

Limitations on coverage coverage for medically necessary expenses of diagnosis and treatment of infertility shall be based on clinical guidelines and the patient's medical history. Clinical guidelines shall be maintained in written form and available to any enrollee. Standards or guidelines developed by the American Society for Reproductive Medicine, the American College of Obstetrics and Gynecology, the Society for Assisted Reproductive Technology, or similar relevant medical societies may serve as a basis for such clinical guidelines. Making, issuing, circulating, or causing to be made, issued or circulated, any clinical guidelines that are based upon data that are not reasonably current or that do not cite with specificity any references relied upon shall constitute an unfair and deceptive act and practice pursuant to section 2 of chapter 93A.

Consistent with Massachusetts anti-discrimination law, coverage for medically necessary expenses of diagnosis and treatment of infertility shall be provided without discrimination based on age, ancestry, color, disability, domestic partner status, gender, gender expression, gender identity, genetic information, marital status, national origin, race, religion, sex, or sexual orientation.

This section shall not be construed to deny or restrict any existing right or benefit to coverage and treatment of infertility or fertility services under an existing law, plan, or policy.

This section shall not be construed to interfere with a medical provider's, physician's, or surgeon's clinical judgment.

SECTION X. Section 4J of chapter 176B, as appearing in the 2022 Official Edition, is hereby amended by striking out the final two sentences and inserting in place thereof the following:-

For purposes of this section, "infertility" means a condition or status characterized by any of the following:

(1) A licensed physician's findings, based on: a patient's medical, sexual, and reproductive history; age; physical findings; diagnostic testing; or any combination of those factors. This definition shall not prevent testing and diagnosis of infertility to establish infertility with or without appropriate exposure to gametes, per the patient's provider.

(2) The need for medical intervention, including, but not limited to, the use of donor gametes, donor embryos, gestational carrier to achieve a live birth either as an individual or with a partner.

(3) The failure to establish a pregnancy or to carry a pregnancy to live birth after unprotected sexual intercourse. For purposes of this section, "unprotected sexual intercourse" means no more than 12 months of unprotected sexual intercourse for a person under 35 years of age or no more than 6 months of unprotected sexual intercourse for a person 35 years of age or older. Pregnancy that does not result in a live birth will not restart the 12-month or 6-month time period to qualify as having infertility.

(4) An impairment of reproductive ability due to factors, including, but not limited to, medical condition, male factor, female factor, combined or unexplained reproductive challenges, as well as genetic disorders or iatrogenic infertility.

Coverage for medically necessary expenses of diagnosis and treatment of infertility shall include, but shall not be limited to: (i) a minimum of six oocyte retrievals and unlimited fresh and frozen embryo transfers, using single embryo transfer when recommended by patient's physician and medically appropriate; (ii) embryo transfer; (iii) artificial insemination; (iv) surgical sperm extraction procedures; (v) third-party reproduction including in vitro fertilization with donor egg, sperm, or embryo or gestational carrier; (vi) procedures necessary to screen or diagnose a fertilized egg before transfer, including, but not limited to, preimplantation genetic testing for aneuploidy, preimplantation genetic testing for chromosome structural rearrangements, and preimplantation genetic testing for monogenic or single gene disorders.

In administering coverage for medically necessary expenses of diagnosis and treatment of infertility, a carrier or participating provider, as those terms are defined in section 1 of chapter 176O, shall not:

(1) impose conditions for eligibility beyond what is provided in the law;

(2) exclude, limit, or otherwise restrict coverage or processing of benefits for fertility medications that are different from those imposed on other prescription medications;

(3) exclude or deny coverage of any fertility services, including medication, based on an individual's participation in fertility services provided by or to any third party. For purposes of this paragraph, "third party" includes: (i) any fresh or cryopreserved oocyte, sperm, or embryo,

183 regardless of the initial coverage source of the donor or the genetic material; and (ii) a gestational
184 carrier that enables an intended parent, member, and/or partner of a member to become a parent.

185 (4) exclude services based on the quantity of the patient's existing cryopreserved oocyte,
186 sperm, or embryos; the provider's discretion will determine if cryopreserved oocyte, sperm, or
187 embryo provides a reasonable chance of success and whether additional fertility services are
188 required;

189 (5) implement any deductible, copayment, coinsurance, benefit maximum, waiting
190 period, or other limitation on coverage that is different from those imposed upon benefits for
191 services not related to infertility;

192 (6) impose limitations on coverage based solely on arbitrary, non-medically based factors
193 including, but not limited to, number of attempts, dollar amounts, or age; or

194 (7) provide different benefits to, or impose different requirements for different groups,
195 based on diagnosis.

196 Limitations on coverage coverage for medically necessary expenses of diagnosis and
197 treatment of infertility shall be based on clinical guidelines and the patient's medical history.
198 Clinical guidelines shall be maintained in written form and available to any enrollee. Standards
199 or guidelines developed by the American Society for Reproductive Medicine, the American
200 College of Obstetrics and Gynecology, the Society for Assisted Reproductive Technology, or
201 similar relevant medical societies may serve as a basis for such clinical guidelines. Making,
202 issuing, circulating, or causing to be made, issued or circulated, any clinical guidelines that are
203 based upon data that are not reasonably current or that do not cite with specificity any references

204 relied upon shall constitute an unfair and deceptive act and practice pursuant to section 2 of
205 chapter 93A.

206 Consistent with Massachusetts anti-discrimination law, coverage for medically necessary
207 expenses of diagnosis and treatment of infertility shall be provided without discrimination based
208 on age, ancestry, color, disability, domestic partner status, gender, gender expression, gender
209 identity, genetic information, marital status, national origin, race, religion, sex, or sexual
210 orientation.

211 This section shall not be construed to deny or restrict any existing right or benefit to
212 coverage and treatment of infertility or fertility services under an existing law, plan, or policy.

213 This section shall not be construed to interfere with a medical provider's, physician's, or
214 surgeon's clinical judgment."